

geha-coverage-policy-trastuzumab.pdf

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Trastuzumab: Herceptin, Herceptin Hylecta, Hercessi, Herzuma, Kanjinti, Ogivri, Ontruzant, Trazimera

Effective Date: 11/10/2024

Revision Date(s): 01/17/2025

Review Date: 9/10/2024

Policy type: Medical Necessity

Line of Business: All GEHA Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Drug Name	Preference	Requires Prior Auth	HCPCS Code	Description
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Trazimera	Preferred	Yes	Q5116	Injection, trastuzumab-qyyp, biosimilar, (Trazimera), 10 mg
Kanjinti	Preferred	Yes	Q5117	Injection, trastuzumab-anns, biosimilar, (Kanjinti), 10 mg
Herceptin	Non- Preferred	Yes	J9355	Injection, trastuzumab, excludes biosimilar, 10 mg
Herceptin Hylecta	Non- Preferred	Yes	J9356	Injection, trastuzumab, 10 mg and hyaluronidase- oysk
Hercessi	Non- Preferred	Yes	Q5416	Injection, trastuzumab-strf (hercessi), biosimilar, 10 mg
Ontruzant	Non- Preferred	Yes	Q5112	Injection, trastuzumab-dttb, biosimilar, (Ontruzant), 10 mg
Herzuma	Non- Preferred	Yes	Q5113	Injection, trastuzumab-pkrb, biosimilar, (Herzuma), 10 mg
Ogivri	Non- Preferred	Yes	Q5114	Injection, Trastuzumab-dkst, biosimilar, (Ogivri), 10 mg

Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents prior to approval of a non-preferred product.

Preferred product requirements apply to new starts ONLY.

Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

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National Comprehensive Cancer Network (NccN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; OR

United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with all components of the product labeling; OR.

Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:

O Clinical Pharmacology (Strong For); OR

O Wolters Kluwer Lexi-Drugs (Level A); OR

Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:

O American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NccN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;.

O Meeting abstracts and case reports are excluded from consideration;.

Non-standard protocols may be approved based on unique clinical circumstances;.

Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network@ (NccN), or indication specific peer-reviewed literature;

In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NccN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

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Herceptin	J9355	Injection, trastuzumab, excludes biosimilar, 10 mg
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Hercessi	Q5416	Injection, trastuzumab-strf (hercessi), biosimilar, 10 mg
Ontruzant	Q5112	Injection, trastuzumab-dttb, biosimilar, (Ontruzant), 10 mg
Herzuma	Q5113	Injection, trastuzumab-pkrb, biosimilar, (Herzuma), 10 mg
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Trazimera	Q5116	Injection, trastuzumab-qyyp, biosimilar, (Trazimera), 10 mg
Kanjinti	Q5117	Injection, trastuzumab-anns, biosimilar, (Kanjinti), 10 mg

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NCCN.org.

Disclaimer

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Date	Updates
11/14/2023	Updated Step Therapy to Table, Added Universal Criteria, Updated References, Updated footer
4/17/2024	Updated Universal Criteria
5/22/2024	Addition of Hercessi
9/10/2024	Annual Review
1/17/2025	Updated HCPCS code for Hercessi, Updated document format to be universally accessible